What decision must leadership make?

HRHS Enterprise AI: From Strategic Decision to Engineering Execution | Fictional Healthcare Technology Advisory Case Study

THE OPPORTUNITY

$640M annual supply spend; on-contract est. 71% vs 85% target

$38M annual denial write-offs; 30% to 50% assumed preventable

11 + 2 proposed AI use cases, plus two pilots running without enterprise review

vs

THE CONSTRAINT

92% of FY2027 discretionary IT capacity already committed

6,800 uncommitted hours vs ~14,000 needed for a governed first phase

4,600 specialist hours needed are committed to the ERP and EHR programs

The decision (Dec 9, 2026, scenario): How should HRHS approach enterprise AI, and what can it actually commit to in FY2027, given that approving a strategy does not create engineering capacity?

What do we know, assume, and need to validate?

Finding	Simulated evidence [S]	Implication	Validation status	Decision consequence
Governance is absent (F-05)	No AI inventory, risk tiering, or evaluation; P-02 ran 5 months with no success measure	Value and risk cannot be compared across 11 proposals	**High within case**	Minimum governance needed under any option
Unresolved PHI vendor risk (F-04)	P-01 vendor data-use terms unreviewed; contract auto-renews Jan 31	HIPAA applies; no violation identified, but terms must be verified before renewal	**Confirm by Day 30 (V-04)**	Act now regardless of option (D-04)
Data readiness is uneven (F-02)	18% of item-master records incomplete; 23% of denials coded "other"	Supply chain usable with remediation; clinical data is not	**Validate Days 30 to 60 (V-02, V-03)**	Start in supply chain; sequence UC-02
Specialist capacity is binding (F-08)	6,800 of 85,000 hours free; 4,600 specialist hours committed to ERP/EHR	Approval does not create capacity	**Validate Days 15 to 20 (V-05)**	Recommendation conditional on validated capacity (D-05)

Read with care: every finding is simulated within a fictional case. In a real engagement, each would be validated (V-01 to V-07) before money or people are committed. Estimates are plus or minus 30%.

Which alternative best serves the organization?

A Targeted, Vendor-Led

\+ Fastest value (4 to 6 months); lowest 3-year cost

– Controls applied product by product; vendor dependency

Capacity 9,000 IT hours (~1,200 specialist)

Exposure $2.55M FY27 | $6.35M 3-yr

Weighted score **3.43**

B Governed, Phased

Provisional recommendation

\+ Reusable controls and shared capabilities lower cost of later use cases

– ~$1.4M more than A over 3 years; needs specialist release

Capacity 14,000 IT hours (4,600 specialist)

Exposure $3.75M FY27 | $7.75M 3-yr

Weighted score **3.52**

C Platform-First

\+ Strongest long-term foundation

– Value deferred to 2028; exceeds envelope

Capacity 30,000 IT hours (~11,000 specialist)

Exposure $7.9M FY27 | $12.3M 3-yr

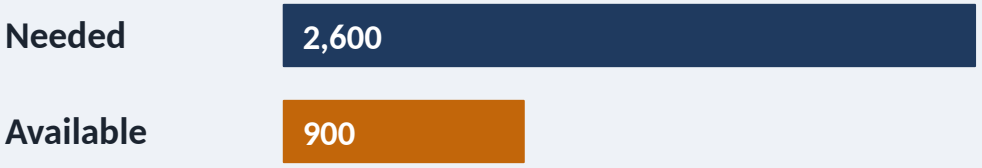
Weighted score **2.51**

The recommendation changes if: HRHS has no pipeline beyond Wave 1, or weights speed over control (A wins); specialist capacity cannot be validated (reduced B, else A); capacity is freed and 15+ use cases are committed (C). All options meet the same mandatory controls; the 0.09 gap is within judgment error.

Can the organization actually deliver the decision?

1 THE CONSTRAINT

1,700 specialist hours unavailable when Wave 1 needs them (Days 1 to 120)



Hours exist in aggregate (14,000 planned). The skills and the delivery window do not: ERP conversion and EHR cutover hold the same specialists. Found in validation on Day 18 and escalated the same day.

2 THE ALTERNATIVES

1. **Sequence Wave 1**
   UC-01 now, UC-02 after ERP go-live: savings arrive ~4 months later

2. **Reallocate engineering capacity**
   Holds the AI schedule, but puts the ERP go-live at risk

3. **Purchase or source externally**
   Adds ~$0.35M and cannot start before ~Day 75

3 THE DECISION

Sequence Wave 1 now. Obtain vendor pricing for a Day 60 decision. Reallocate ERP capacity only after written confirmation of acceptable impact.

How will leadership know whether execution is working?

Day 0
G0 Authorize
ELT

Days 15 / 20
Capacity allocated, then validated
CIO/CTO + TPM

Day 30
G1 Governance baseline
Steering

Day 60
G2 Design and readiness
Steering + CISO

~Day 65
G3 Pilot go/no-go
Risk Board + product owner

Day 90
G4 Scale, revise, or stop
CFO + ELT

Delivery readiness (G3)

- Acceptance criteria approved
- Controls MC-01 to MC-06 evidenced
- Support runbook and rollback
- Pilot users trained

Business value

- On-contract spend vs 71% baseline
- Output acceptance rate
- Net denial recovery collected
- Finance-validated, never summed

Escalation thresholds

- Capacity unavailable in window: 1 day
- Gate at risk > 2 weeks
- Variance > 10% or $100K
- Patient-data incident: 24 hours

Proceed: criteria met, controls conform

Pivot: evidence favors Option A

Slow: capacity short: sequence scope

Stop: criteria unmet after one revision

AI informs. Humans decide. Leaders own.